NCCP 00379 · clinician draft

PCV chemotherapy · 42-day cycle

PCV · 42 days NCCP 00379: adjuvant treatment of grade II glioma after radiotherapy, or palliative treatment of recurrent high-grade glioma. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 29: vincristine only

Active medicines: Vincristine

Emetogenic risk: Minimal

Medicine	Dose / route	When
Constipation prevention	Complete the locally prescribed laxative and dose	With vincristine exposure; review bowel function before each infusion NCCP PCV supportive care recommends prophylaxis. Severe pain, vomiting or inability to pass stool/wind requires urgent assessment, not automatic extra laxatives.

Additional phase-specific support

- No routine antiemetic prophylaxis is required for vincristine alone; prior symptoms or other medicines may require an individual rescue plan.
- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep any prescribed seizure medicine and steroid plan separate from the PCV calendar. Report new weakness, tingling, gait change or constipation before vincristine. A medicine review is needed before adding any treatment for symptoms.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00379 v4 · p4 · NCCP oral-SACT antiemetics V6: Table 5 p8; timing p4; interactions p11

NCCP regimen-specific support · Sources checked 2026-11-05 · SACTCheck v0.84.38 · Regimen-level drafts; prescriber and oncology-pharmacy verification required